

CPT Code Clinical Guide

When to use each code · What to document · SNF Therapy · Medicare Part B · 2026

LOW VALUE

EVAL

MED VALUE

HIGH VALUE

Value:

Evaluations				
Code	Discipline	Use When	Document	Tips
97162 / 97163	PT	Initial PT evaluation on admission or significant change in status	Prior level of function, objective measures, functional deficits, skilled need, plan of care	Use 97163 (High) when patient has 3+ comorbidities, complex history, or significant functional decline
97166 / 97167	OT	Initial OT evaluation — ADL/IADL deficits, cognitive or UE impairment	Occupational profile, ADL performance, ROM/strength, cognitive status, home environment	Use 97167 (High) for complex cognitive, neuro, or multi-system presentations
92523	ST	ST eval when cognitive-communication or language deficits are present	Cognitive-comm screen, language processing, standardized test scores, functional communication	Pair with 92610 if swallowing is also a concern — bill separately
92610	ST	Clinical swallowing evaluation (CSE) — not instrumental	Oral motor exam, trial consistencies, aspiration risk, diet recommendation	Can be billed same day as 92523 if both language and swallowing are evaluated

High-Value Treatment				
Code	Discipline	Use When	Document	Tips
97533	OT	Sensory processing deficits — stroke, TBI, neuro dx with sensory integration impairment	Sensory-specific deficits in evaluation, SI treatment activities documented, patient response	Highest-value OT code. Requires appropriate diagnosis. MPPR: bills at 100%, bumps others to 50%
97530	PT OT	Functional task training — transfers, mobility tasks, dynamic balance, real-world activities	Specific functional task, patient response, skilled cueing or grading required	Most versatile high-value code. Use when therapy goes beyond simple exercise into function
92526	ST	Dysphagia treatment — swallowing rehab, oral motor strengthening, compensatory strategies	Target swallowing phase, techniques used, diet consistency, patient response, safety	Highest-value ST treatment code. Requires prior swallowing eval to support billing
92507	ST	Speech, language, voice, fluency treatment — aphasia, dysarthria, voice disorders	Target area, treatment technique, standardized measure or functional baseline, patient response	Core ST treatment code. Use for language, voice, or fluency goals — not swallowing

Mid-Value Treatment				
Code	Discipline	Use When	Document	Tips
97112	PT OT	Balance retraining, proprioception deficits, coordination training, gait retraining	Specific neuromuscular deficit addressed, treatment technique, objective response	Pair with 97110 or 97530 for patients with combined strength + balance deficits
97535	OT	ADL/IADL training — dressing, bathing, grooming, meal prep, home management tasks	Specific ADL task, level of assist before/after, adaptive equipment, skilled instruction	Core OT code for SNF. Documents functional progress clearly — use consistently
97110	PT OT	Therapeutic exercise — strengthening, ROM, endurance when active patient participation required	Specific muscle groups, sets/reps/resistance, progress from prior session	Requires skilled need — document why HHA or restorative staff cannot perform independently
97116	PT	Gait training — weight shifting, step sequencing, assistive device training, stairs	Gait deviations addressed, distance/assist level, device used, skilled cueing provided	Do not use for simple ambulation that does not require skilled therapy intervention
97129	OT ST	Cognitive intervention — attention, memory, problem-solving, executive function (first 15 min)	Cognitive domain targeted, standardized task or activity, patient performance and response	97129 = first 15 min. Bill 97130 for each additional 15 min in the same session

CPT Code Clinical Guide

When to use each code · What to document · SNF Therapy · Medicare Part B · 2026

LOW VALUE EVAL MED VALUE HIGH VALUE

Value:

Code	Discipline	Use When	Document	Tips
97140	PT	Manual therapy — joint mobilization, soft tissue mobilization, manual stretching	Specific joint or tissue, technique used, pre/post ROM or pain response	Requires skilled hands-on technique — document why passive ROM alone is insufficient

Specialty / Other				
Code	Discipline	Use When	Document	Tips
97542	PT OT	Wheelchair fitting, mobility training, pressure relief education	WC components addressed, seating assessment findings, patient/caregiver training provided	Bill only when skilled assessment or training is required — not for routine WC use
97760	PT OT	Initial orthotic fitting and training — AFO, UE splint, knee brace	Device type, fit assessment, skin check, functional use training provided	Use 97763 for all follow-up orthotic management visits after initial fitting
97032	PT	Neuromuscular electrical stimulation (NMES) — muscle re-education, post-surgical atrophy	Muscle group, parameters, patient response, functional carryover documented	NMES for muscle re-ed is covered. TENS for pain management has very limited SNF coverage

Documentation reminder: Every code billed must demonstrate skilled need — why this service requires a licensed therapist and cannot be performed by a HHA or restorative aide. Document objective measures, patient response, and progress toward goals every session.